

## Department of Cancer Pathology

copy No.

Date:

## Examination: Histopathological examination

Internal invoice No.

Cost of diagnostic procedure

Examination No.:

Patient: XXX

PESEL: XXX

Age:

Gender: M

Material: **Multiple organ resection – left breast with axillary tissues**

Unit in charge:

Physician in charge:

Material collected on:

Material received on:

Expected time of examination: **up to 8 working days**Clinical diagnosis: **Cancer of the left breast. Upper outer quad.**

1CD-0-3  
 carcinoma, ductal and micropapillary  
 (order to highest) 8507/3  
 Site: breast, NOS C50.9 for 4/15/11

Examination performed on:

Results of immunohistochemical examination:

Estrogen receptors found in over 75% of neoplastic cell nuclei. Progesterone receptors found in over 75% of neoplastic cell nuclei. HER2 protein stained with HercepTest™ by DAKO.

Positive reaction in invasive cancerous cells ( Score = 3+ )

Compliance validated:

Examination performed on:

Macroscopic description:

**Left breast sized 17.5 x 14 x 3 cm removed along with axillary tissues sized 8 x 12 x 3 cm and a skin flap of 13 x 5 cm. Tumour sized 3.5 x 2.5 x 2.0 cm found on the boundary of outer quadrants, located 4.7 cm from the lower boundary, 0.15 cm from the base and 0.8 cm from the skin. Lymph nodes from 0.2 to 2.8 cm.**

Microscopic description:

**Carcinoma ductale invasivum (partim Carcinoma micropapillare) – NHG2 (3+2+2/11 mitoses/10 HPF - visual area: 0.55mm).**

**Foci of carcinoma ductale in situ (DCIS) present within the tumour (micropapillary and cribrate type with medium nuclear atypia.**

**Mamilla sine laesionibus). Glandular tissue showing gynaecomastia. Axillary lymph nodes:**

**Metastases et micrometastases carcinomatosae in lymphonodis (No XXV/XXVIII). Infiltratio capsulae lymphonodorum et telae perinodalis.**

Histopathological diagnosis:

**Carcinoma ductale invasivum partim carcinoma micropapillare invasivum. Invasive ductal and partially micropapillar carcinoma of the left breast. Metastases et micrometastases carcinomatosae in lymphonodis axillae (No XXV/XXVIII). Cancer metastases and micrometastases in axillary lymph nodes (NHG2; pT2; pN3a).**

Compliance validated h.

CONTACT YOUR DOCTOR WITH THIS REPORT!

UUID: 6C97C157-05A0-45CE-91E9-3DC8946EFEDE  
 TCGA-DB-A1XS-01A-PR

Redacted



| Criteria                       | Yes         | No                                  |
|--------------------------------|-------------|-------------------------------------|
| Diagnosis Discrepancy          |             | <input checked="" type="checkbox"/> |
| Primary Tumor Site Discrepancy |             | <input checked="" type="checkbox"/> |
| HIPAA Discrepancy              |             | <input checked="" type="checkbox"/> |
| Prior Malignancy History       |             | <input checked="" type="checkbox"/> |
| Dual/Synchronous Primary       |             | <input checked="" type="checkbox"/> |
| Case # (circle)                | QUALIFIED   | DISQUALIFIED                        |
| Reviewed by                    | for 4/15/11 |                                     |